



OFFICIAL

Armadale Health Service Emergency Response Plan

Code Black

Code Black - Personal Threat / Patient Self Harm
Code Black Alpha - Infant / Child Abduction
Code Black Bravo - Active Armed Offender

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Document Control

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Amendments	
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Dec 2025	Replaced active shooter with active armed offender throughout the document. Added a section on weapons.

Creations, Amendments and Deletions

Amendment			Entered	
Version	Number	Date	Signature	Date
1	Original version	Mar 2009	Unknown	Mar 2009
2	- Major changes including; - Ward and departmental changes - Changed references from WestPlan Health to State Health Emergency Response Plan	Dec 2016	M Hayward	December 2016
3	Review and update	Sep 2018	M Hayward	Sep 2018
4				
5	Review and update; - Replaced active shooter with active armed offender throughout the document. Added a section on weapons. - Format change.	October 2025	December 2025	December 2025

Abbreviations

AAC	Assembly Area Coordinator
AH	Armadale Hospital (Armadale Hospital excluding mental health)
A/H	After Hours
AHS	Armadale Health Service (Armadale Hospital including mental health and off site facilities)
AIIMS	Australasian Inter-service Incident Management System
AKG	Armadale Kalamunda Group (Armadale Health Service and Kalamunda Hospital)
ALS	Advanced Life Support
ANUM	Associate Nurse Unit Manager
AS / NZS	Australian Standard / New Zealand Standard
ASAP	As Soon As Possible
BCP	Business Continuity Plan
BGA	Break Glass Alarm (Manual Call Point)
BLS	Basic Life Support
BMS	Building Management System
CBRN	Chemical, Biological, Radiological & Nuclear
CCTV	Closed Circuit Television
CEM	Coordinator Emergency Management
CIMS	Clinical Incident Management System
CNM	Clinical Nurse Manager
CNS	Clinical Nurse Specialist
CSSD	Central Sterilisation Supply Department
DFES	Department of Fire & Emergency Services
DCS	Director Clinical Services
DOH	Department of Health
DOHA	Department of Health & Ageing
DONM	Director of Nursing & Midwifery
DPMU	Disaster Preparedness & Management Unit (Department of Health)
ED	Emergency Department
ED DO	Emergency Department Duty Officer
EDIS	Emergency Department Information System
EM	Emergency Management
EMC	Emergency Management Committee
EOC	Emergency Operation Centre
ERT	Emergency Response Team
ESC	Emergency Scene Coordinator
FIP	Fire Indicator Panel
GCS	Glasgow Coma Scale
HazMat	Hazardous Materials
HIC	Hospital Incident Commander
HICS	Hospital Incident Command System
HMA	Hazard Management Agency
HOD	Head of Department
HNM	Hospital Nurse Manager
HRSO	Hospital Radiation Safety Officer

HRT	Health Response Team
HSS	Health Support Services
ICS	Incident Control System
ICU	Intensive Care Unit
ID	Identification
ISO	International Standards Organisation
IMT	Incident Management Team
IPM	Infection Prevention & Management
KH	Kalamunda Hospital
LPG	Liquid Petroleum Gas
LRSO	Local Radiation Safety Officer
MET	Medical Emergency Team
MAU	Medical Admissions Unit
MIMMS	Major Incident Medical Management Support
MSDS	Material Safety Data Sheet(s)
NUM	Nurse Unit Manager
OCCO	On Call Clinical Officer (Department of Health)
OCDO	On Call Duty Officer (Department of Health)
PA	Public Address System
PABX	Private Automatic Branch eXchange
PCA	Patient Care Assistant
PMS	Pop-up Messaging System
PPE	Personal Protective Equipment
PSS	Patient Support Services
RDD	Radiation Dispersal Device
RITH	Rehabilitation In the Home
RMO	Resident Medical Officer
RRCD	Recognising & Responding to Clinical Deterioration
SAIR	Staff Accident & Incident Report
SDS	Safety Data Sheet
SEMC	State Emergency Management Committee
SES	State Emergency Service
SHICC	State Health Incident Coordination Centre
SJA	St John Ambulance
SMS	Short Message Service
SWP	Safe Work Procedures
TOR	Terms of Reference
UPS	Uninterrupted Power Supply
WAPS / WAPOL	Western Australian Police Service
WebPAS	WEB patient administration system
WHS	Work Health and Safety
WIP	Warden Intercom Point

1. Introduction

The hospital is required to provide a safe working environment for its employees, and the public have an expectation that their healthcare will be delivered in a safe environment. As part of meeting these expectations, the East Metropolitan Health Service employs Security Officers at Armadale Hospital.

Activating a Code Black is a method where staff members who feel that their personal safety, or the safety of others, is under threat, and can get a prompt response from appropriately trained personnel. Under no circumstances should staff, patients or visitors place themselves at risk.

The correct management of an aggressive, agitated, violent or threatening person can decrease assaults or harm to patients, staff and visitors. Aggression can be classified as either:

- Aggressive non-violent
- Violent Person – unarmed, assault
- Violent Person – armed (including hold up or hostage)

A **Code Black - Personal Threat**: must also be used to raise the alert for a suicide or self-harm attempt, an armed robbery, or a hostage situation.

A **Code Black - Alpha**: must be used in every actual or suspected event of an infant / child abduction, or attempted abduction.

A **Code Black - Bravo**: must be used when a person armed with a firearm or other weapon and is actively engaged in attempting to cause serious harm to multiple people in a populated location.

2. Authority to Declare

Any staff member that fears for their safety, or the safety of others, may declare a Code Black – Personal Threat, or activate a Duress Alarm. As outlined below, there are several methods to obtain assistance, and depending on the actual situation encountered, some may be more appropriate than others.

3. Coordination of Code Black Response

All declared Code Black – Personal Threat incidents on the Armadale Hospital site will be coordinated through the Security Control Room. In the following situations, the activation of the Emergency Operations Centre (EOC) is required:

- i. Code Black - Alpha
- ii. Code Black - Bravo
- iii. Code Black Suicide Threats where:
 - a. the situation is not resolved within five minutes of the arrival of the first Emergency Response Team Member
 - b. additional resources are required to assist in the situation, such as extra orderlies, mental health/psychiatry staff.
 - c. the person is recognised as an inpatient.
- iv. Armed, siege or hostage incidents
- v. A protracted and uncontrolled incident
- vi. An incident where staff, patients or the public are significantly injured and require

immediate medical treatment.

- vii. An incident where large numbers of staff, patients or the public are involved.

The Hospital Nurse Manager (HNM) is to immediately inform the Senior Responder on Call (SROC)

/ Hospital Incident Commander (HIC) who will activate the Incident Management Team (IMT). Off-site situations may require staff to dial 000 and request immediate Police presence. If the SROC decides to step up the Emergency Operations Centre (EOC) the HNM will send out a message to the IMT via Esendex.

4. All Clear and Stand Down

The Security Officers can call an 'All Clear' when the situation is controlled. When the Emergency Scene Coordinator (ESC) (or Security Officer 1 in the absence of the ESC) is satisfied that all issues within the scene are attended to, the Security Control Room Officer can declare a Stand Down. If there is likely to be a delay, the issuing of a partial stand down will allow the release of Emergency Response Team members not required on scene to return to normal duties.

If the EOC has been activated, the Hospital Incident Commander, or their delegate, is responsible for declaring the Stand Down. Recovery activities can then commence.

5. Documentation and Investigation

Security Officers are responsible for ensuring an electronic log entry is completed for all declared Code Blacks. The Security Supervisor will review each incident and forward to the set distribution list. Where an investigation is required, the most appropriate unit will coordinate the investigation and prepare a report. If a Code Black meets the criteria listed above and the EOC is activated, then an ETHANE report is to be completed for that phase of the incident and entered into WebEOC.

6. Boundaries

All areas of the Armadale Hospital site are included in this plan. See separate plan for Kalamunda Hospital.

7. Aggressive Non-Violent Person

When do you call for assistance?

- Any verbal or physical threat (actual or perceived)
- Noisy and disruptive (*Where a patient becomes noisy and disruptive within a clinical area, due to a medically related issue, it is expected that this will be dealt with as a clinical matter. Security may be called to assist where staff feel threatened*)

7.1 Action Plan – aggressive non-violent person

- Remove yourself from danger area.
- Summons assistance by
 - ☞ Contact Security directly – ext. *4333 or Page, and/or
 - ☞ Activate Duress Alarm where available.
 - ☞ When you contact security, or are contacted following a Duress Alarm activation, give the following:
 - Exact location

- Description of problem (where possible)
 - Your name
- Maintain your safety.
- Where possible isolate the perpetrator
- On arrival Security Officers are to be briefed by Clinical Staff and then work with the Clinical Staff to manage the situation appropriately.
- Other available staff are to follow the directions of Clinical Lead / CNS / NUM until the situation is contained (i.e. keep others safe, contact other staff as required)
- Once the situation is contained or made safe, the CNS / NUM (local area or Hospital Nurse Manager) and the Nursing Shift Coordinator (or Manager / Supervisor in Non-Clinical areas) will ensure the following occurs:
 - ☞ Staffing backup is arranged to relieve staff involved if required
 - ☞ Provide details on how to access to the Employee Assistance Program to all staff involved either directly or indirectly, where required
 - ☞ Completion of incident forms (CIMS, CHIOR) and required statements by staff involved
 - ☞ Staff are to be offered the opportunity to report the incident to the police, where appropriate.

8. Violent Person – Unarmed, Assault

If a person becomes violent towards another person or property, the following procedure is to be implemented:

8.1 Action Plan – violent person-unarmed assault

- Stay out of danger if you are not directly involved. Leave the area if safe to do so.
- If a discreet alert is required, use Duress Alarm where available
- If safe to do so **PHONE '55'** and **STATE**:
 - ☞ "Code Black – Violent Person"
 - ☞ Give exact location of incident
 - ☞ Describe the situation
 - ☞ Your name and title
- If you can't retreat, remain where you are until help arrives
- Observe offender's: -
 - ☞ Physical appearance (scars, beards, tattoos, piercings)
 - ☞ Speech
 - ☞ Clothing
 - ☞ Behaviour
- Where relevant / possible observe offender's vehicle: -
 - ☞ Vehicle registration
 - ☞ Type
 - ☞ Colour
 - ☞ Number of occupants
 - ☞ Direction of travel
 - ☞ Other details of note (dents, stickers)
- When assistance arrives, follow the directions of Security and/or Emergency Scene Coordinator
 - When safe to do so, all staff to complete independently the Offender Description Form (available in action cards) where required
 - Request all staff and other witnesses to remain until statements collected by WA

Police, unless they require medical attention

- When the situation is resolved, the Emergency Scene Coordinator is to ensure the following occurs:
 - ☞ Liaise with Management to arrange staff relief if required
 - ☞ Liaise with MET / Emergency Department to arrange medical attention for those involved, where required.
 - ☞ Provide details on how to access the Employee Assistance Program to all staff involved either directly or indirectly, where required
 - ☞ Ensure the completion of statements and incident forms (CIMS, CHOIR) by staff involved
 - ☞ Offer staff the opportunity to report the incident to the police, where appropriate.

9. Violent Person – Armed (Including Hold Up or Hostage)

If a person becomes violent towards another person or property or is identified to be carrying a weapon and/or threatens to use a weapon, the following procedure is to be implemented.

Where a firearm is mentioned to the switchboard operator, the WA Police are to be called immediately on 000. If it is any other form of weapon, security will attend and assess the need for police attendance.

**Where a firearm is involved the WA Police will be notified immediately.
They will take control of the situation on arrival**

9.1 Action Plan – violent person-armed (including hold up or hostage)

- Obey the offender's instructions, but only do what you are told and nothing more. Do not volunteer information
- Stay out of danger if you are not directly involved. Leave the area if safe to do so
- If a discreet alert is required, use Duress Alarm where available
 - If safe to do so, **PHONE '55'** and **STATE**:
 - ☞ "Code Black – Violent Person - Armed"
 - ☞ Give exact location of incident
 - ☞ Describe the weapon and/or situation
 - ☞ Your name and title
- If you can't retreat, remain where you are until help arrives
- Observe offender's: -
 - ☞ Physical appearance (scars, beards, tattoos, piercings)
 - ☞ Speech
 - ☞ Clothing
 - ☞ Behaviour
- Where relevant / possible observe offender's vehicle: -
 - ☞ Vehicle registration
 - ☞ Type
 - ☞ Colour
 - ☞ Number of occupants
 - ☞ Direction of travel
 - ☞ Other details of note (dents, stickers)
- When assistance arrives, follow the directions of Security and/or Police

- Preserve the area until WA Police give clearance to return
- When safe to do so, all staff to complete independently the Offender Description Form (available in action cards)
- Request all staff and other witnesses to remain until the WA Police attend, unless they require medical attention.
- When the situation is resolved, Emergency Scene Coordinator is to ensure the following occurs:
 - ☞ Liaise with Management to arrange staff relief if required.
 - ☞ Liaise with MET / Emergency Department to arrange medical attention for those involved, where required
 - ☞ Provide details on how to access to the Employee Assistance Program to all staff involved either directly or indirectly, where required
 - ☞ Ensure the completion of statements and incident forms (CIMS, CHOIR) by staff involved

10. Suicide Attempt / Deliberate Self Harm

When a patient verbally expresses suicidal intentions, contact the admitting team Resident or Registrar, who may:

1. Call the Psychiatry Consultation Liaison Team via switchboard for review and assistance for in-patients (during normal hours).
2. Call the on-call Psychiatry Registrar for inpatients after hours and during normal hours if the Consultation Liaison cannot be contacted.

For persons that are not inpatients, or identifiable as such - DIAL '55'.

When a person is actively attempting to commit suicide or engaging in self-harm, implement the following procedure.

10.1 Action Plan – suicide attempt / deliberate self-harm

- Maintain a safe distance (at least 6 metres)
- Summons assistance by:
 - ☞ If a discreet alert is required, use Duress Alarm where available
 - ☞ **PHONE '55' and STATE:**
 - ☞ "Code Black - Suicide/Self Harm Attempt"
 - ☞ Exact location
 - ☞ Method of harm threatened/occurring.
 - ☞ Your name and title
- Communicate calmly with the person from a safe distance (usually a minimum of 6 metres) if you feel comfortable to do so while waiting for the Emergency Response Team to arrive
 - Remove people not involved from the area
 - On arrival the Emergency Response Team will:
 - ☞ Assess the situation
 - ☞ Plan response, and make requests for assistance as required, through the EOC / ESC for:
 - On-call Psychiatry Registrar
 - Psychiatric Consultation Liaison CNS
 - Medical Emergency Team
 - Police

- Ambulance Service

☞ Implement plan

- When the WA Police arrive, they will assess and take overall control of the situation. In consultation with the Emergency Scene Coordinator, Hospital Incident Commander and Psychiatric Team, an appropriate course of action will be determined, which may include the Police Negotiation Team
- On containment of the situation, access treatment for person as required. The treating team and the Psychiatry Registrar / Consultation Liaison team will document the treatment plan and follow-up, as required. If the incident has involved a patient, they are to be returned to the ward, or taken to the Emergency Department, as directed by the Psychiatric Team and/or Emergency Scene Coordinator. If the incident involved a person that is not a patient of the hospital, they are to be taken to the Emergency Department for further assessment
- Once the situation is resolved, the Emergency Scene Coordinator ensures the following occurs:
 - ☞ Arrange staffing backup to relieve staff involved if required
 - ☞ Provide details on how to access to the Employee Assistance Program to all staff involved either directly or indirectly, where required
 - ☞ Ensure the completion of statements and incident forms (CIMS, CHOIR) by staff involved
- See Post-Incident Management section in this plan for further details of debriefing if required.

11. Infant or Child Abduction

Newborn abduction from a hospital is an infrequent, devastating event for both the families and the staff involved. The incidence of newborn abduction appears to be an increasing event and health services should give special consideration to the security of newborn babies. The issue should be addressed in the context of risk management and hospital security and protective measures should be implemented for babies, their families and staff. Consideration should also be given to older babies and young children on the paediatric ward. Areas at risk within AKG are as follows;

- Maud Bellas Ward
- Birthing Suites
- Neonatal Nursery
- Campbell paediatric ward
- Emergency Department

11.1 Typical abductor profile (National Centre for Missing & Exploited Children, 2018)

- Usually a female of childbearing age who appears pregnant
- Most likely compulsive; most often relies on manipulation, lying and deception
- Frequently indicates she has lost a baby or is incapable of having one
- Often married or cohabitating; companion's desire for a baby or the abductors desire to provide her companion with "his" baby may be the motivation for the abduction
- Usually lives within the community where the abduction takes place

- Frequently initially visits nursery and maternity units at more than one health care facility prior to the abduction; asks detailed questions about procedures and the maternity floor layout; frequently uses a fire exit stairwell for her escape; and may also try to abduct from the home setting
- Usually plans the abduction, but does not necessarily target a specific infant; frequently seizes any opportunity present to abduct a baby
- Frequently impersonates a nurse or other allied health care personnel
- Often becomes familiar with health care staff members, staff member work routines and victim parents
- Often demonstrates a capability to provide care to the baby once the abduction occurs, within her emotional and physical abilities

11.2 Abduction from the home setting

- More likely to be single while claiming to have a partner
- Often targets a mother whom she may find by visiting health care facilities and tries to meet the target family
- Often plans the abduction **and** brings a weapon, although the weapon may not be used
- Often impersonates a health care or social services professional when visiting the home

There is no guarantee an infant abductor will fit this description.

11.3 Typical neonate profile

- Usually, seven days old or younger
- Is perceived by the abductor as hers
- Is the same race/skin colour as the abductor or the abductors partner

Where an infant or child up to 16 years of age is abducted, or an abduction attempt is in progress, the following procedure is to be implemented to minimise the risk of harm to the infant or child and give the best chance of recovery.

11.4 Action Plan – infant / child abduction

- DO NOT attempt to stop the suspect, as they may harm the infant or child
- **PHONE ‘55’ and STATE:**
 - ☞ “Code Black – Alpha”
 - ☞ Child / Infant abduction
 - ☞ Give exact location of incident
 - ☞ Your name and title
- Switchboard are to immediately activate their Code Black Alpha lockdown lever on the wall in switchboard or in the afterhours ED reception area by pulling the lever towards the horizontal position

Figure 1 – Code Black Alpha Lockdown Lever



- This will immediately lock all external doors on the first and ground floors until de-activated by security
- Maud Bellas are also to activate their Code Black Alpha lockdown lever in the Maud Bellas staff base area by pulling the lever towards the horizontal position
- If possible, get a staff member to follow the suspect at a safe distance
- All area wardens are to follow their action cards
- If the location of the suspect is unknown, begin a search, and prevent people from leaving local area until help arrives
- Begin compiling description of child / infant, to assist the response team
- Make note of any persons suspected to be involved: -
 - ☞ Identity (if known)
 - ☞ Physical appearance (scars, beards, tattoos, piercings)
 - ☞ Speech
 - ☞ Clothing
 - ☞ Behaviour

Note: Where a child is missing, with no suspicion of abduction, contact Security on extension *4333.

12. Active Armed Offender

Places of mass gathering (PMG) including hospitals are prime target for religious and political extremists, as well as disgruntled or mentally impaired individuals. Active armed offender attacks have occurred and will continue to occur in hospitals and other crowded places.

Australia is not immune from active armed offender events. Despite regulation of access to firearms, the use of firearms is still considered one of the most likely methods of attack in Australia. Several significant events have occurred at government buildings, shopping centres, universities and public spaces in the last 25 years. The Port Arthur, Hoddle Street, Strathfield, Queen Street and Lindt café incidents show that active armed offender events are a real and persistent threat to the community.

While most active armed offender incidents are different from one attack to another, there are certain common features. Typically, active armed offender incidents happen quickly and are usually over within 10 - 15 minutes. Active armed offenders will continue to harm victims until any of these three events occur – the active armed offender runs out of ammunition, they are confronted by law enforcement personnel or the active armed offender commits suicide.

Active armed offender incidents cannot be entirely prevented. However, there must be plans in place to protect staff, patients and visitors in a hospital to protect them from a threat with potentially severe and deadly consequences.

As per OD 0593/15 hospitals and health services are required to have in place procedures guiding the prevention, preparedness, response and recovery to an active armed offender incident. Health facilities are responsible for identifying designated staff requiring mandatory training in Code Black Bravo procedures.

The aim of any response to an active armed offender event is to minimise the offender's access to potential victims. People not able to immediately move to an area of safety should shelter in place. Where possible hide behind a strong object, jam doors shut if possible, turn lights off and turn phones and pager to silent or off. Without exposing yourself to further danger report the event by calling '55' and stating Code Black Bravo. If discovered by the offender, attempting to negotiate can assist.

12.1 Definition

OD 0593/15 Feb 2015 outlines the code used for an Active Armed Offender event. This code shall be known as Code Black Bravo. An active armed offender is "a person(s) armed with firearms(s) or other weapons who is actively engaged in killing or attempting to cause serious harm to multiple people in populated location."

12.2 Aims and Objectives

Active armed offenders need freedom of movement to access victims, therefore minimising the offender's access to potential victims is the primary aim of the Code Black Bravo plan.

Time + Freedom of movement = Increased casualties

The objective of the Code Black Bravo plan is to ensure the health service has adequate procedures in place to respond including prevention, preparedness, response and recovery to an active armed offender event.

This includes:

- initiating immediate response activities.
- minimising the duration of the incident.
- restricting the offender(s) movements.
- moving people from danger.
- preventing people from entering the scene; and
- assisting Police to locate and contain the armed offender

12.3 Scope

This plan will cover prevention, preparedness, response and recovery.

12.4 Prevention

- Deterrent measures such as security officers and patrols, physical and electronic barriers
- Restricting access to vulnerable areas of the hospital
- Active utilisation of Closed Circuit Television (CCTV) to monitor vulnerable areas of the hospital. Whilst CCTVs are available, these will not be monitored when security officers are not in the office
- Established systems to detect, report and respond to intruders
- Established systems to promote staff awareness and reporting of suspicious behaviour

12.5 Preparedness

Training and exercise requirements

- Basic awareness and response options will be provided in corporate induction, mandatory update days and through emergency training updates delivered by Emergency Management Coordinators/Managers
- Role specific training will be provided to security officers at formal training and in-service education
- Exercising of the Code Black Bravo plan will be incorporated into other exercise opportunities at least once every two years. Any exercises shall include external agencies such as WA Police, DFES and St John Ambulance as active participants

12.6 Staff awareness

- Action plan for “Code Black bravo” to be included in emergency training annually
- Highlight the seriousness of the active armed offender threat and provide localised action plan for staff through targeted messages cascading through local Coordinators/Managers/Area Wardens and delivered to staff. Typically, messages may be cascaded through general or targeted emails or internal memos, or through materials presented at team meetings or huddles

12.7 Informing patients and visitors

- Action plans of what to do in the unlikely event of code black bravo to be included in admission pack
- Community Advisory Group to be involved

12.8 Probable entrance

Most likely entrance of the active armed offender includes the Main Entrance, Galliers, Day therapy, Mental Health and ED waiting room.

12.9 Weapons

Firearms

Historically, active armed offender incidents in Australia have involved the use of long arm weapons (rifles and shotguns), although the use of handguns has become more prominent over the past decade. Firearm ownership restrictions introduced by the government following the Port Arthur massacre significantly reduced the number of semi-automatic weapons in the country. However, some licensed firearms, and firearms acquired through illegal means, continue to pose a genuine threat to the public. Domestic and international experience emphasises that well-planned and coordinated attacks using high powered firearms remain an enduring threat.

Prohibited Weapons

Laws have been changed to make our community safer by reducing the number of items in the community that are easily concealed, easily carried, and are designed to kill or injure people. These laws make it an offence for a person to have prohibited weapons, unless exempted in special and limited circumstances. Although illegal, many prohibited weapons can still be purchased without license and exist in significant numbers in the community. An active armed offender has relatively easy access to these weapons, including edged weapons such as large knives, machetes and swords.

Improvised Weapons

Improvised weapons are commonly used by attackers when they are unable to obtain conventional weapons such as firearms and/or prohibited weapons.

Anything can be an improvised weapon if it is carried with the intention of hurting someone or if it is being carried in a threatening way. Any common or every day item such as a knife, bottle, screwdriver, baseball bat, or even a motor vehicle or a large rock can be an improvised weapon.

A motivated individual can easily use such items to kill or inflict serious injury. Improvised weapons are only limited by an individual's imagination. It should be noted they could also be used to defend against an active armed offender as a last resort.

12.10 Action Plan – active armed offender

Local area involvement

Advice for individuals is based on the three basic principles of run, hide and engage.

Run and hide

The **absolute priority** for any staff caught up in the local area of an **active armed offender** event is to move to an area of safety away from the incident. Staff should consider the following points to enable their escape:

- Take cover initially – get down low and hide behind large items, such as cabinets or desks
- Turn your mobile phone and pager to silent
- Then, attempt to leave the area as soon as possible if safe to do so
- Leave belongings behind, but take your mobile phone with you
- Know your exit route and maintain cover as best you can whilst leaving the area
- Warn other people as quietly as you can
- Do not activate the duress alarm
- Do not activate code blue
- Do not activate the fire alarms
- Do not congregate at assembly areas
- If unable to leave, for in-patient ward areas, consider closing fire doors and attempt to barricade doors with beds to prevent the active armed offender from entering
- Consider getting patients to lie on the floor and close doors to rooms
- Consider activating door release buttons if doors are locked and no swipe card available
- Do not activate the Emergency Operations Centre or the Incident Management Team during the incident

Engage

- Used as a last resort and when your life is in imminent danger.
- If unable to run or hide, attempt to engage with the active armed offender by:
 - Acting as aggressively as possible towards the active armed offender
 - Throwing items and improvising weapons
 - Yelling
 - Committing to your actions

12.11 Activating code black bravo

- Do this only if safe to do so
- After running and hiding, **dial 55** using a hospital phone or 9391 2000 using your mobile phone – State “Code Black Bravo” and your location.
- **Call “000”** to inform police of active armed offender if possible
 - Useful information to provide include – location of active armed offender, number and type of weapons held by the offender and number of offenders

12.13 When police officers arrive

- Remain calm
- Stay low
- Put your hand on your head and shout “Staff member, staff member, staff member”.
- Avoid making quick movements toward officers such as holding on to them for safety

- Avoid pointing, screaming or yelling

12.14 Neighbouring areas

- **Dial 55** using a hospital phone or 9391 1199 using your mobile phone – State “Code Black Bravo” and your location
- Currently lock down capability in ICU, operating theatre and woman’s and children’s ward
- Move towards “safety refuge areas” if safe to do so
 - Ward staff will perform risk assessment on individual wards and identify safety areas within respective wards. These areas are known as “safety refuge rooms”, which allow staff to hide in case of an active armed offender.
 - If in ward areas, this is likely to include medication rooms
- Turn the lights off and close individual patient rooms if it is safe to do so.
- A hiding place should:
 - Be out of the active armed offenders view
 - Provide protection if shots are fired in your direction
 - Not trap you or restrict your options for movement
- To prevent the active armed offender from entering your hiding place:
 - Lock the door
 - Block the door with heavy furniture, for example, patient beds
- It is unlikely that there will be time to evacuate patients. Patients are advised to stay in their rooms, shut the doors and hide. This is with the exception of having a clear exit route nearby, where patients may choose to run out of the hospital.

12.15 Position statement on patients

The aim of this active armed offender plan is to provide the maximum good for the greatest number of patients. Wherever possible, the risk of harm to patients will be minimised. Mobile patients are encouraged to hide in the patient room or take refuge within safety refuge rooms. Lights will be turned off to reduce visibility for the active armed offender and individual patient rooms will be shut to maximise protection to patients. Further information will also be available within patient admission packs.

12.16 Reception

- On receiving the **first 55** “code black bravo”, the receptionist will **call “000”** to notify WA Police of the active armed offender incident as a priority
- PA announcement to inform staff and patients of code black bravo. “Active armed offender, (location). Take immediate action to hide or escape”
- Base page security and Hospital Nurse Manager code black bravo (location)

12.17 Hospital Nurse Manager

- Upon hearing the active armed offender announcement on the PA system, dial 000 and inform WA Police
- Upon hearing the active armed offender announcement on the PA system, the Hospital Nurse Manager should contact the SROC immediately
- If the offender is in the main entrance, the Hospital Nurse Manager should exercise extreme caution and proceed to the fire panel in ED if safe to do so. Where the nurse manager is able to get to the fire panel, further active armed offender announcements will be made manually through the fire panel if unable to be made from switchboard

12.18 Security

- Security officers are not to engage the offender
- One security officer will remain in the security control room, while another security officer will proceed to reception area if it is safe to do so
- The security officer in the control room will identify the last location of the active armed offender via CCTV and communicate this to the security officer in the reception area
- The security officer in the reception area will then update staff regarding the last location of the active armed offender via the PA system
- If active armed offender is in the main entrance and the nurse manager is at the fire panel, the security officer will communicate this with the nurse manager regarding the last location of the active armed offender
- If security officers are at a code black or not in the control room, one security officer will proceed to the control room, while the other will proceed to the reception area

12.19 Recovery

- Activate the Emergency Operations Centre
- Activate the Incident Management Team
- Inform the State Health Incident Coordination Centre via the on call duty officer (9328 0553)
- Activate the Armadale Health Service Business Continuity Plan
- Radio communication will be used to coordinate movement of patients/staff
- Transport injured persons to ED; or dispatch by ambulance to other emergency departments if necessary
- Leave any deceased persons due to it being a forensic crime scene. This is with the exception of needing to move a deceased body to allow a person who is alive to get out.
- Arrange critical incident debriefing for staff and patients
- Manage the media
- Conduct an impact assessment regarding business disruption
- Refine planning & training as appropriate

Personal Description of Offender Form

Notes For Compilation By Each Witness

To be compiled immediately after incident. A separate form is required for each person/offender. Place a tick as applicable. If answer is unknown, write 'Unknown' against the heading. Do not consult others during compilation. Security will collect forms and hand to the Police.

NAME or NICKNAMES USED.....				SEX	Male	Female
APPROXIMATE AGE				ETHNIC ORIGIN		
HEIGHT.....				WEIGHT.....		
COMPLEXION	Fair	Dark	Pale	BUILD	Thin	Stout
	Fresh	Ruddy	Suntan		Medium	Nuggetty
	Pimply			VOICE	Clear	Loud
ACCENT.....					Thick	Slangy
POSTURE	Erect	Stooped	Slouchy	SPECTACLES	Colour	
WALK	Quick	Springy	Slow		Shape	
	Limp	Pigeon toed			Thick Glass	Tinted
HAIR	Colour... ..	Crewcut		MOUSTACHE-BEARD	Type.....	
	Straight	Wavy	Bald	DISGUISE		
	Curly	Thick	Long			
EYES	Colour			HANDS	Size.....	Hairy
	Size				Callused	Soft
	Intense Stare	Squint			Nails/Missing/Deformed fingers	
EARS	Size			GLOVES	Type	
	Shape				Colour	
NOSE	Size			JEWELLERY	Describe	
	Shape			SCARS OR MARKS – tattoos, scars, discolouration, describe location fully		
LIPS	Size					
	Shape					
TEETH	Good	Uneven	Spaced	WEAPON TYPE		
	Bad	Protruding	Missing			
CLOTHING – Including hat, tie, shirt, coat, trousers, Dress, skirt, sweater and shoes.....				METHOD AND DIRECTION OF ESCAPE		
METHOD OF OPERATION: What did the offender do, say, touch, carry, etc.....				Make of car		
NAME				Model of car		
ADDRESS.....				Colour		
SIGNATURE.....Date:.....				Registration		
				Number of Vehicles used:		

Description of Abducted Infant / Child Form

To be completed by employee caring for infant/child, with assistance of parent/s if possible
Security will collect forms and hand to the Police.

Person completing form:

SURNAME: _____ FIRST NAME: _____

Position Title: _____

Ward Area: _____ Phone No.: _____

Date: _____ Time child last seen or attended: _____

SIGNATURE: _____

Description of Infant / Child:

SURNAME: _____ First Name(s): _____

DOB: _____ Date of admission: _____

UMRN: _____

SEX: Male Female RACE: _____

HEIGHT: Metres: _____ cm: _____ Feet: _____ inches: _____

WEIGHT: kg: _____ stone/lbs: _____

BUILD: thin solid medium obese

HAIR: (colour) _____ soft short straight thick
medium wavy thin
long curly bald

EYES: (colour) _____

(size) _____

Medical Condition:

Ability to vocalise:

Other Distinguishing Features:

Scars or Birthmarks - describe in full:

Clothing

Last Meal / drink / feed

Action Cards

Code Black	Person Identifying the Code Black	Action Card 1
Personal Threat / Self-Harm / Suicide		
1.	Remove yourself, and others, from immediate danger if able to do so. Under no circumstances should staff, patients or visitors place themselves at risk	
2.	Raise the alert using the following methods: Summon Security directly in person, or via ext. *4333 OR Activate the duress alarm OR Phone '55' and state 'Code Black'	
3.	If safe to do so, attempt to de-escalate the situation, whilst maintaining a safe distance. The acronym LASSIE may assist; L – LISTEN to the person – remain calm and non-confrontational A – ACKNOWLEDGE their problem S – SEPARATE them from others S – SEAT them I – INDICATE options to resolve issues E – ENCOURAGE them to look at solutions	
4.	Provide information to the Security Officers on their arrival	
5.	Inform your Area Warden / manager	
6.	Follow the instructions of the Security Officers / Emergency Scene Coordinator and Area Warden	
Code Black Alpha - Child / Infant Abduction		
A1.	Do not attempt to stop the suspect, as they may harm the child or infant	
A2.	Activate CBA lockdown lever (Maud Bellas staff base only)	
A3.	Phone '55' and state 'Code Black Alpha' (area)	
A4.	Inform Area Warden / Shift Coordinator / Midwifery Unit Manager	
A5.	Ring security on *4333 and give them a description of the abductor and which direction they were heading	
A6.	If possible / safe get a staff member to follow the suspect at a safe distance	
A7.	Begin compiling a description of the offender and the child / infant	

A8.	Brief Emergency Scene Coordinator when they arrive
A9.	Assist Police when they arrive
Code Black Bravo - Active Armed Offender	
B1.	Take cover initially then attempt to leave the area as soon as possible, if safe to do so
B2.	Report the event by calling '55' and stating 'Code Black Bravo'
B3.	Maintain cover as best you can whilst leaving the area
B4.	Warn other people as quietly as you can
B5.	Take note of what is happening – location, direction they are heading and number of shooters
B6.	Ensure mobile phone / pages are turned to silent
Stand Down / Recovery Actions	
7.	Complete an incident log into Choir / CIMS (when able)
8.	Attend debrief

Code Black	Switchboard Operator	Action Card 2
All Hours		
When a Code Black is declared, if the caller has not provided sufficient details, use prompts to identify: • the exact location • brief details of the emergency • the caller's identity		
1.	Code Black Personal Threat	
	• Receive a '55' call via the IIC • Activate Emergency Response Team via group page local ID 001 • <u>Example message:</u> 'Code Black MAU person threatening staff' • If a clinical area, send same message to relevant NUM • If you are requested to contact the WA Police dial '000', or '131 444' if stated as non-urgent • Await confirmation or requests for further assistance	
	Code Black Alpha - Child / Infant Abduction	
	• Receive a '55' call via the IIC • Immediately activate CBA lockdown lever • PA announcement - Code Black Alpha (location), activate plan (if after hours make PA announcement from main switchboard) • Activate Code Black Alpha Emergency Response Team (base page local ID 167) • Await further instructions	
	Code Black Bravo - Active Armed Offender	
	• If safe to do so; • Receive a '55' call via the IIC • PA announcement - Code Black Bravo. "Active Armed Offender, (location). Take immediate action to hide or escape" • Call WA Police on '000' • Activate Code Black Bravo Emergency Response Team (base page local ID 167) • Await further instructions	

2.	Contact the following via mobile to alert them to the Code Black emergency: <u>During Office Hours (Monday – Friday, 07:30hr to 16:00hr)</u> • Hospital Nurse Manager on 0414 276 425 • Security on ext. *4333 • Relevant manager <u>Out of Hours (Monday – Friday 16:00hr to 07:30hr, weekends and Public Holidays)</u> • After Hours Nurse Manager on 0414 276 425 • Security on ext. *4333
3.	• The Senior Responder on Call / Hospital Incident Commander may request an alert to be sent to the 'local ID 001' group page, requesting attendance at the Emergency Operations Centre (EOC) • The EOC (Ext. 2599) will be in contact if additional assistance is required from Switchboard Operator
4.	Stand Down / Recovery Actions A Stand Down instruction will need to be sent via pager to the 'Local ID 001' group page where requested - Switchboard will be notified of the Stand Down by the Hospital Incident Commander or Emergency Scene Coordinator <u>Example message:</u> '<i>Stand Down Code Black MAU</i>
5.	Attend debrief

Code Black		Area Warden (Emergency Scene)	Action Card 3
		Ward Shift Coordinator Designated Area Warden Most senior person present	
Reporting to: Emergency Scene Coordinator			
1.	On becoming aware of a Code Black, assume a coordination role for your area		
2.	Ascertain type of threat and location		
3.	Ensure '55' has been called and/or Duress Alarm has been activated.		
4.	Don Area Warden tabard and gather action cards		
5.	Ensure all staff, patients and visitors are removed from danger, as far as practicable		
6.	Where the offender is armed – follow all instructions, whilst trying to remain at least 6 metres from them		
Personal Threat / Self-Harm / Suicide			
7.	In a Self-Harm or Suicide Attempt : remove people from immediate danger, and try to establish a dialogue with the person making the threats, whilst maintaining a minimum of 6 metres from that person		
Code Black Alpha - Child / Infant Abduction			
8.	In a Code Black Alpha : • Ensure the peripheral lockdown lever has been activated • Ensure '55' has been dialled • Initiate a local search if offender / child believed to still be in area • Allocate a staff member to stay with the parent/s, • Ensure security are given a description of the offender via phone		
Code Black Bravo - Active Armed Offender			
9.	In a Code Black Bravo : If safe, quietly warn people in your area and either leave the area, or shelter in place if unsafe to leave		
10.	Liaise with and brief the Security Officers and Emergency Scene Coordinator on their arrival		
11.	Act on instructions from the Security Officers and Emergency Scene Coordinator		
Stand Down / Recovery Actions			
12.	Await 'Stand Down' order		
13.	Liaise with the Emergency Scene Coordinator and Security Officer/s to develop a management plan		

14.	Attend debrief
15.	Complete an incident log into Choir / CIMS (when able)

Code Black	Area Warden (Other wards / areas)	Action
	Ward Shift Coordinator Designated Area Warden	Card 4
Reporting to: Emergency Scene Coordinator		
1.	On becoming aware of a Code Black, assume a coordination role for your area	
Code Black Alpha - Child / Infant Abduction		
2.	In a Code Black Alpha : - Assign a staff member to man each external ward exit immediately - Allocate staff to search your department and adjacent corridors - Report back to Emergency Scene Coordinator when search complete	
Code Black Bravo - Active Armed Offender		
3.	In a Code Black Bravo : If safe, quietly warn people in your area and either leave the area, or shelter in place	
4.	Liaise with and brief the Security Officers and Emergency Scene Coordinator on their arrival	
5.	Act on instructions from the Security Officers and Emergency Scene Coordinator	
Stand Down / Recovery Actions		
6.	Await 'Stand Down' order	
7.	Attend debrief	

Code Black		Emergency Scene Coordinator Rostered Hospital Nurse Manager	Action Card ERT 1
Reporting to: Senior Responder on Call			
Personal Threat			
1.	Proceed directly to the scene of the incident Note: Security will take the lead role in a Code Black		
2.	Obtain a briefing from: - Area Warden - Security Officers		
3.	Provide clinical input and advice to assist Security Officers as required		
4.	If required, assist Security Officers to develop an action plan		
5.	Ensure scene has been secured from visitors, bystanders and other staff		
6.	Maintain a safe distance from the hazard		
Self-Harm / Suicide			
i.	Obtain a description/history of incident from Security or other staff member/s present		
ii.	If Security are not involved in talking to person making threats, attempt to verbally engage them Note: You should maintain a distance of at least 6 metres from the person		
iii.	When able, inform the EOC (if stood up) or SROC of status, and if escalation of response is required, e.g.; - Psychiatry Liaison - Police service - Code Blue		
Code Black Alpha - Child / Infant Abduction			
A1.	Proceed directly to the scene of the incident		
A2.	Ensure the Area Warden has initiated a '55' call and activated lock down		
A3.	Initiate a local search if offender / child believed to still be in area, or escalate to the Emergency Operations Centre (EOC) if activated		
A4.	Ensure a staff member has been allocated to stay with the parent/s / guardian		

A5.	Liaise with the Area Warden and Security Officers to have a staff or family member familiar with the child / infant attend the Security Control Room
A6.	Maintain liaison role with the WA Police at the scene
A7.	Ensure SROC / Hospital Incident Commander is kept informed at all times
A8.	Ensure protection of the crime scene, isolate the area where the infant was abducted
Code Black Bravo - Active Armed Offender	
B1.	Upon hearing the active armed offender announcement on the PA system, or paging system, dial 000 and inform WA Police
B2.	Upon notification of active armed offender contact the SROC immediately
B3.	If the armed offender is in the main entrance, the Hospital Nurse Manager should exercise extreme caution and proceed to the fire panel in ED if safe to do so. Where the nurse manager is able to get to the fire panel, further active armed offender announcements will be made manually through the fire panel if unable to be made from switchboard
B4.	Await further tasking from the Hospital Incident Commander
Stand Down / Recovery Actions	
7.	Requests for any assistance or resources to be communicated to the EOC when stood up. Only STAND UP the EOC post active armed offender event
8.	Ensure staff and patient welfare are attended to, and appropriate plans put in place as required
9.	Ensure staff are debriefed and relevant Choir / CIMS forms are completed
10.	Enter into WebEOC

Code Black		Security Officer 1	Action Card
			ERT 2
Reporting to: Emergency Scene Coordinator / HIC			
Personal Threat / Self-Harm / Suicide			
1.	Security Officer 1 shall proceed to the scene of the emergency		
2.	Where possible obtain a briefing from staff present at the scene		
3.	Assess the scene and decide on a plan of action in liaison with the Emergency Scene Coordinator (if appropriate / available)		
4.	Advise the Emergency Scene Coordinator when situation is contained		
Code Black Alpha - Child / Infant Abduction			
A1.	Attend the main front entrance and man the door. Obtain pertinent details from the Area Warden of child / infant, and any persons suspected to be involved (via mobile phone) and broadcast to other Security Officers over radio as soon as possible		
A2.	Liaise with the Emergency Scene Coordinator		
A3.	Continually receive updated information		
A4.	Request resources / assistance as required		
Code Black Bravo - Active Armed Offender			
B1.	Security Officers are not to engage the armed offender		
B2.	Proceed to the main entrance / reception area and wait for WA Police if safe to do so		
B3.	Liaise with Security Control Room and Emergency Scene Coordinator		
B4.	Where able, attempt to determine location, direction of travel and number of armed offenders – from the Security Control Room CCTV officer and relay information to staff via the PA system		
B5.	If the armed offender is in the main entrance and the Emergency Scene Coordinator is in the fire control room, the Security Officer will communicate this to the Emergency Scene Coordinator to be broadcasted over the PA system from the fire control room		
B6.	If Security Officers are attending a Code Black or not in the control room, one Security Officer will proceed to the control room while the other will proceed to		

	the main front entrance as at point B2
B7.	Prevent other people from entering the area where the armed offender is believed to be located
B8.	Assist other people to escape the area where the armed offender is believed to be located or in the direction they are heading
Stand Down / Recovery Actions	
6.	Where required liaise with the Emergency Scene Coordinator to develop a management plan in conjunction with relevant stakeholders
7.	'All Clear' to be communicated to the Security Control Room (in liaison with the Emergency Scene Coordinator where available)
8.	Ensure the security log sheet has been completed in Sentry
9.	Attend debrief

Code Black		Security Officer 2	Action Card
			ERT 3
Reporting to: Emergency Scene Coordinator / HIC			
Personal Threat / Self-Harm / Suicide			
1.	Security Officer 2 shall proceed to the scene of the emergency		
2.	Where possible obtain a briefing from staff present at the scene		
3.	Assess the scene and decide on a plan of action in liaison with the Emergency Scene Coordinator (if appropriate / available)		
4.	Advise the Emergency Scene Coordinator when situation is contained		
Code Black Alpha - Child / Infant Abduction			
A1.	Attend the Galliers front entrance and man the door. Obtain pertinent details from the Area Warden of child / infant, and any persons suspected to be involved (via mobile phone) and broadcast over radio as soon as possible		
A2.	Liaise with the Emergency Scene Coordinator		
A3.	Continually receive updated information		
A4.	Request resources / assistance as required		
Code Black Bravo - Active Armed Offender			
B1.	Security Officers are not to engage the armed offender		
B2.	Proceed to the main reception area and wait for WA Police if safe to do so		
B3.	Liaise with Security Control Room and Emergency Scene Coordinator		
B4.	Where able, attempt to determine location, direction of travel and number of armed offenders – from the Security Control Room CCTV officer and relay information to staff via the PA system		
B5.	If the armed offender is in the main entrance and the Emergency Scene Coordinator is in the fire control room, the Security Officer will communicate this to the Emergency Scene Coordinator to be broadcasted over the PA system from the fire control room		
B6.	If Security Officers are attending a Code Black or not in the control room, one Security Officer will proceed to the control room while the other will proceed to the main front entrance as at point B2		

B7.	Prevent other people from entering the area where the armed offender is believed to be located
B8.	Assist other people to escape the area where the armed offender is believed to be located or in the direction they are heading
Stand Down / Recovery Actions	
6.	Where required liaise with the Emergency Scene Coordinator to develop a management plan in conjunction with relevant stakeholders
7.	'All Clear' to be communicated to the Security Control Room (in liaison with the Emergency Scene Coordinator where available)
8.	Ensure the security log sheet has been completed in Sentry
9.	Attend debrief

Code Black		Security Officer 3	Action Card
			ERT 4
Reporting to: Emergency Scene Coordinator / Logistics Officer			
Personal Threat / Self-Harm / Suicide			
1.	Security Officer 3 shall proceed to the scene of the emergency		
2.	Where possible obtain a briefing from staff present at the scene		
3.	Assess the scene and decide on a plan of action in liaison with the Emergency Scene Coordinator (if appropriate / available)		
4.	Advise the Emergency Scene Coordinator when situation is contained		
Code Black Alpha - Child / Infant Abduction			
A1.	Attend the security control room and monitor CCTV – particularly at exits. Obtain pertinent details from the Area Warden of child / infant, and any persons suspected to be involved (via mobile phone) and broadcast over radio to other Security Officers as soon as possible		
A2.	Liaise with the Emergency Scene Coordinator		
A3.	Continually receive updated information		
A4.	Request resources / assistance as required		
A5.	When staff or family members/s arrive to view CCTV, assist them to view footage		
Code Black Bravo - Active Armed Offender			
B1.	Attend the security control room and liaise with security 1 and 2		
B2.	Monitor CCTV		
B3.	Where able, assist Security 1 & 2 to determine location, direction of travel and number of armed offenders		
B4.	Ensure WA Police have been notified, and that Security Officer 1 or 2 has been dispatched to meet them at the front entrance		
B5.	Liaise as appropriate		
Stand Down / Recovery Actions			

6.	Where required liaise with the Emergency Scene Coordinator to develop a management plan in conjunction with relevant stakeholders
7.	Ensure the security log sheet has been completed in Sentry
8.	If the EOC has not been activated: Once the scene has been declared 'All Clear' by the Security Officer/s and/or Emergency Scene Coordinator, contact switch on 55 and declare a stand down
9.	Provide CCTV footage and other evidence to WA Police, as per policies and procedures
10.	Attend debrief

Code Black		PCA / Orderly	Action Card
		Page 026	
			ERT 5
Reporting to: Emergency Scene Coordinator			
Personal Threat / Self-Harm / Suicide			
1.	Proceed as quickly as possible to the location shown on pager and report to the Hospital Nurse Manger (Emergency Scene Coordinator)		
2.	Other tasks as directed by the Emergency Scene Coordinator		
Code Black Alpha - Child / Infant Abduction			
A1.	Proceed directly to the rear door along the CSSD corridor. This door is manned by the PCA until stood down		
A2.	If requested, proceed to the EOC and assist the Hospital Incident Commander as directed		
Code Black Bravo - Active Armed Offender			
B1.	Proceed to the EOC (once stood up – post event)		
B2.	Deliver ERT vests and two-way radios to the scene as directed by the Hospital Incident Commander		
B3.	Remain at the EOC and assist by the Hospital Incident Commander as directed		
Stand Down / Recovery Actions			
3.	Tasks as directed by the Emergency Scene Coordinator / Hospital Incident Commander		
4.	Attend debrief		

Code Black		Engineer	Action Card
			ERT 6
Reporting to: Emergency Scene Coordinator / HIC			
1.	If requested, Respond to the scene of the incident		
2.	Provide technical assistance to the Emergency Scene Coordinator / Hospital Incident Commander / Security Officer / WA Police		
3.	Provide access as required <u>Or</u> Assist with searching plant rooms and / or roof tops, as required		
4.	Where required, assess damage to fittings, plant and infrastructure		
Stand Down / Recovery Actions			
5.	Arrange for repair of damage as required		
6.	Attend debrief		

Code Black		Senior Responder on Call		Action Card	
		Rostered SROC 24/7			
				IMT 1	
Reporting to: Executive Director – Hospital Incident Commander					
Responsibilities: Initial leadership of the incident				Sign	Time
Initially assume the role of the Hospital Incident Commander and manage the hospital's response to the emergency, until relieved by the Executive Director or nominated executive					
Immediate Actions				Sign	Time
1.	On notification of the incident from the Emergency Scene Coordinator, decide whether the Emergency Operations Centre (EOC) should be stood up and proceed directly to the EOC (page 4 outlines situations where the EOC should be stood up)				
Code Black Alpha - Child / Infant Abduction				Sign	Time
1a.	If Code Black Alpha activate the EOC, and liaise with the Security Control Room; • open HIC box • don grey HIC tabard • confirm hospital lockdown • confirm PCAs, ward staff and security are in position at exits • activate a Grid Search if required • escalate to WA Police Service • activate the Incident Management Team • Inform the State Health Incident Coordination Centre (SHICC) via the On Call Duty Officer (OCDO) on - o 9328 0553 • assist Security to access additional resources • access the Pop-Up Message System • advise the Hospital Incident Commander (Executive Director) • Ensure the crime scene is protected • advise EMHS Communications Manager				
Code Black Bravo - Active Armed Offender				Sign	Time

1b.	If Code Black Bravo activate the EOC (post event), and <u>in liaison with</u> Security Control Room: • assist Security to access additional resources • escalate to WA Police Service • activate the Incident Management Team • advise the Hospital Incident Commander (Executive Director)		
	• don grey HIC tabard • Inform the State Health Incident Coordination Centre (SHICC) via the On Call Duty Officer (OCDO) on - o 9328 0553 • Place MET and St John Ambulance on standby • Access the Pop-Up Message System • Radio communication will be used to coordinate movement of patients/staff • Ensure injured are transported to ED; or dispatch by ambulance to other emergency departments if necessary (in liaison with the SHICC) • Leave any deceased persons due to it being a forensic crime scene. The exception is to access live casualties • advise EMHS Communications Manager		
2.	On arrival to EOC; • Don Hospital Incident Commander grey tabard • Log into WebEOC using the generic logon and log in as the Hospital Incident Commander in the EOC details tab • Turn up the volume on the MERN base radio • Turn on simocoXd hand held radio • Ensure all key activities, actions and decisions are documented in WebEOC and on the Samsung Flip and communicated with the Liaison Officers • Inform Emergency Response Team, via two-way radio, that the EOC is operational		
3.	Ask ERT what information is currently available		
4.	Confirm Incident Management Team are responding		
Intermediate / Extended Actions		Sign	Time
5.	Request a situation report, if not received within 5 minutes		
6.	Confirm that the ERT vests and two-way radios have been dispatched to the scene with a PCA		
7.	Ensure a threat assessment is being undertaken at the scene, and liaise with DFES and / or WA Police as appropriate		

8.	Consider likely impact to hospital services and/or geographical areas, based on information available to this time		
9.	Once situation report is obtained, and impact assessed, consider level of escalation (where required)		
	1. Continue management whilst keeping the Executive Director informed		
	2. Request Executive Director to attend EOC (consider IMT activation in liaison with ED). At this time the Executive Director will become the HIC.		
	3. Stand down if all clear is given by the WA Police and/or ESC / Security, and no other issues on the site		
10.	Assess and determine the need to: • Notify surrounding areas: consider use of Pop-Up Messaging; and • Provide additional assistance / resources		
11.	Obtain specialist advise from the Engineer / Coordinator Emergency Management where required		
12.	Consider other areas that may require evacuation		
13.	If IMT is activated refer to the Action Cards for further actions. Consider covering key actions initially until full team arrive		
14.	Where a briefing or discussion is required, utilise the hospital seminar room		
Stand Down / Recovery Actions		Sign	Time
15.	Activate the Armadale Health Service BCP if required. Conduct an impact assessment regarding business disruption		
16.	Where staff are no longer required for the response, individuals may be stood down		
17.	Ensure all areas involved in the Code Black are notified of the stand down		
18.	Phone Switchboard on '55' and request 'Stand Down' message to be sent to the ERT group page		
19.	Liaise with the Emergency Scene Coordinator regarding staff and patient welfare, and ensure plans are put in place to manage this where required		

20.	Ensure a debrief is arranged and relevant Choir / CIMS forms are completed		
21.	Manage the media		
22.	Refine planning, lessons learned and training as appropriate		

Code Black		Hospital Incident Commander		Action Card	
		Senior Responder on Call			
		Executive Director		IMT 2	
Reporting to: EMHS Chief Executive Officer					
Responsibilities: Leadership of the incident				Sign	Time
Manage the hospital’s response to a Code Black in situations where the EOC is activated and the IMT is stood up. Ensure all actions of the SROC (Action Card IMT 1) are complete					
• All IMT members report to the HIC					
Immediate Actions				Sign	Time
1.	When notified of Code Black incident by Senior Responder on Call (SROC): - Consider full IMT activation - Proceed to the Emergency Operations Centre (EOC)				
2.	On arrival at the EOC: - Receive a briefing from the SROC - Access HIC “Box” - HIC grey tabard to be handed over from SROC - Log into WebEOC using the generic logon and log in as the Hospital Incident Commander in the EOC details tab				
3.	Confirm details of the Code Black:				
	Type of threat				
	Location of incident				
	Is police attendance required? – If yes, are they attending?				
	Is Psychiatry required? – If yes, are they attending?				
	Have any areas been evacuated, or require evacuation?				
	Does access to the area need to be restricted?				
	What impact is this incident having on hospital operations?				
	Is ambulance diversion required?				
	Commence WebEOC entry				
4.	On arrival of IMT members give a briefing on: - The nature of the problem - Any immediate critical issues - The initial plan of action - Ensure all key IMT roles are filled - Consider using the ETHANE SITREP framework Direct Incident Management Team to provide status reports in 15 minutes				

5.	If it is a significant incident inform: • Chief Executive Officer (EMHS) • Dept of Health Duty Officer on 9328 0553		
6.	Consider hospital – wide notification of incident – direct the Liaison Officer to complete		
7.	Ensure all key activities, actions and decisions are documented in WebEOC and on the Samsung Flip and communicated with the Liaison Officers		
8.	As the spokesperson for the hospital, the HIC will respond to phone calls received from: • the Minister for Health; • the Director General, Health; and • the Chief Executive Area Health Services		
Intermediate Actions		Sign	Time
9.	• Complete an Impact Assessment		
	• Services and facilities affected and for how long		
	• Availability of alternate accommodation		
	• The impairment or cancellation of services		
	• Likely assistance required from State Health Incident Coordination Centre, other hospitals or services		
	• Effect of any ongoing investigations e.g. forensics, arson, coroner		
	• Instruct the Liaison Officer to document the assessment		
10.	Direct Incident Management Team to provide status reports as required		
11.	Ensure staff and public are kept informed during incident – instruct Liaison Officer and EMHS Communications Officer to complete updates via global emails etc		
12.	Complete regular briefings for the Incident Management Team		
13.	Where a private briefing or discussion is required, utilise break out offices or use Hospital Seminar Room for larger briefings		
Extended Actions		Sign	Time
14.	Handover to subsequent shift		
15.	Complete briefings at intervals as decided and publicised		

16.	Ensure staff and public are kept informed during incident – instruct Liaison Officer and EMHS Communications Officer to complete		
Stand Down / Recovery Actions		Sign	Time
17.	Review the impact assessment completed at item 8		
18.	Consider stand down of staff members that are no longer required for the response		
19.	Consider activation of Business Continuity Plan/s		
20.	Ensure staff and public are kept informed during recovery – instruct Liaison Officer and EMHS Communications Officer to complete		
21.	Authorise Stand Down when appropriate		
22.	Ensure a hot debrief is held for the personnel involved		

Code Black	Planning Officer Director of Nursing & Midwifery	Action Card IMT 3
Reporting to: Hospital Incident Commander		
Responsibilities: Staffing, welfare and patient flow	Sign	Time
The Planning Officer is the person responsible to the HIC for staffing, welfare and patient flow in support of the incident. Staff that report to this role are as follows; • Planning Staff Outgoing Patient Flow (Nurse Manager, Patient Flow) • Planning Staff Disaster Admissions (Coordinator of Nursing, Patient Flow) • Planning Staff Mental Health Liaison (Coordinator of Nursing, Mental Health)		
Immediate Actions	Sign	Time
1. On activation of the Incident Management Team (IMT) proceed directly to the Emergency Operations Centre (EOC)		
2. On arrival at the EOC: • Receive a briefing from the HIC • Access Planning Officer 'Box' • Don Planning Officer blue tabard • Log into WebEOC using the generic logon and log in as the Planning Officer in the EOC details tab • Commence WebEOC entry • Ensure your planning staff have arrived		
3. Brief the following areas/staff (as required):		
• Nursing Administration		
• Allied Health		
• Human Resources		
• Patient Information Manager		
• Medical Records Manager		
4. Liaise with the Hospital Incident Commander, Operations Officer and Logistics Officer to determine what Planning tasks need to be undertaken		

5.	Liaise with other areas under your command regarding resources (human and physical) required to manage the incident. Areas under command of the Planning Officer are as follows; • Nursing Administration • Allied Health • Medical Records • Patient Information		
	• Human Resources • Bed Management • Patient Tracking • Discharge Area • Relative Reception • Staff Welfare / Clergy • Volunteers		
6.	Liaise with Medical Resources and/or Nursing Administration (where required) to ensure staffing levels are adequate to manage the emergency		
7.	Ensure all key activities, actions and decisions are documented in WebEOC and on the Samsung Flip and communicated with the Liaison Officers		
Intermediate Actions		Sign	Time
8.	Continue to liaise with Operations and Logistics Officers, and areas under your command regarding resource requirements		
9.	Ensure areas under your command continue to receive briefings at regular intervals, and obtain status reports as required		
Extended Actions		Sign	Time
10.	Handover to subsequent shift		
11.	Continue to liaise with Operations and Logistics Officers, and areas under your command regarding resource requirements and managing staff fatigue		
12.	Continue to ensure people under your command are receiving regular briefings		
Stand Down / Recovery Actions		Sign	Time
13.	Ensure that workloads have returned to a pre-internal emergency or acceptable level.		

14.	Ensure plans are in place regarding the ongoing management of patient and/or staff issues, including staff fatigue, following stand down		
15.	Upon Stand Down, ensure that areas under your command are informed of the Stand Down order		
16.	Attend debrief as required		

Code Black		Planning Staff – Outgoing Patient Flow	Action Card	
		Nurse Manager Patient Flow / Hospital Nurse Manager	IMT 4	
Reporting to: Planning Officer				
Responsibilities: Outgoing patient flow			Sign	Time
The outgoing patient flow role manages and coordinates all outgoing patient movement inclusive of inpatient discharges, transfers and decanting. This is done in conjunction with Surgical Patient Flow and Disaster Admissions roles.				
Immediate Actions			Sign	Time
1.	On activation of the Incident Management Team (IMT) proceed to the Emergency Operations Centre (EOC) and obtain a briefing. The Emergency Scene Coordinator role will now be withdrawn.			
2.	On arrival at the EOC: • Receive a briefing from the Planning Officer • Access Planning Officer 'Box' • Don Planning Staff – Outgoing Patient Flow blue tabard • Log into WebEOC using the generic logon and log in as Planning Staff in the EOC details tab • Commence WebEOC entry			
3.	Liaise with Planning Staff – Disaster Admissions and Operations Staff – Surgical Patient Flow to determine bed requirements and staffing of those beds			
4.	Provide a briefing to the following regarding bed requirements and the decanting plan:			
	• Nurse Unit Managers			
	• Heads of Departments			
	• Team Leaders / Managers			
5.	Request the Nurse Unit Managers to supply the following information to you: - Current vacant / available beds (report within 5 minutes) - Estimated potential discharges and transfers (report within 15 minutes) if appropriate			

6.	Ensure all key activities, actions and decisions are documented in WebEOC and on the Samsung Flip and communicated with the Liaison Officers		
Intermediate Actions		Sign	Time
7.	Review hospital decanting plan in order to assess its effectiveness, and make changes as required		
8.	Review the flow of Patients through the hospital, and recommend changes as required		
9.	Liaise with the Nurse Unit Managers, Discharge Coordinator and other relevant staff for updates, and provide additional briefings		
10.	Provide Nurse Unit Managers with regular updates of the situation.		
11.	Ensure staffing plan is in place for Patient Flow for subsequent shifts, if required		
Extended Actions		Sign	Time
12.	Handover to subsequent shift		
13.	Conduct regular staff briefings		
14.	Review the flow of patients through the hospital, and recommend changes as required		
Stand Down / Recovery Actions		Sign	Time
15.	Liaise with Planning Staff – Disaster Admissions and Operations Staff – Surgical Patient Flow to ensure that beds can remain staffed		
16.	Resume normal duties having informed persons contacted by you that the ‘Stand Down’ has been issued.		
17.	Attend debrief as required		

Code Black		Planning Staff – Disaster Admissions	Action Card
		Coordinator of Nursing, Patient Flow	IMT 5
Reporting to: Planning Officer			
Responsibilities: Disaster admissions			Sign
			Time
The disaster admissions role manages and coordinates all disaster admissions into the Health Service			
Immediate Actions			Sign
			Time
1.	On activation of the Incident Management Team (IMT) proceed to the Emergency Operations Centre (EOC) and obtain a briefing.		
2.	On arrival at the EOC: • Receive a briefing from the Planning Officer • Access Planning Officer 'Box' • Don Planning Staff – Disaster Admissions blue tabard • Log into WebEOC using the generic logon and log in as Planning Staff in the EOC details tab • Commence WebEOC entry		
3.	Liaise with Planning Staff – Outgoing Patient Flow and Operations Staff – Surgical Patient Flow to determine bed requirements and staffing of those beds		
4.	Provide a briefing to the following regarding bed requirements; • Nurse Unit Managers • Heads of Departments • Team Leaders / Managers		
5.	Request the Nurse Unit Managers to supply the following information to you: - Current vacant / available beds (report within 5 minutes) - Estimated potential discharges and transfers (report within 15 minutes) if appropriate		
6.	Proceed to the affected area, if safe to do so and identify all potential casualties. Liaise with the Operations Staff in relation to relocation of patients to ED and surgery		
7.	Liaise with the Operations Officer re patients requiring transfer to a tertiary hospital		
8.	Ensure all key activities, actions and decisions are documented in WebEOC and on the Samsung Flip and communicated with the Liaison Officers		

Intermediate Actions		Sign	Time
9.	Provide Nurse Unit Managers with regular updates of the situation.		
Extended Actions		Sign	Time
10.	Handover to subsequent shift		
11.	Conduct regular staff briefings		
12.	Liaise with ED to continually review the number of patients affected		
Stand Down / Recovery Actions		Sign	Time
13.	Liaise with Planning Staff – Outgoing Patient Flow and Operations Staff – Surgical Patient Flow to ensure that beds can remain staffed		
14.	Resume normal duties having informed persons contacted by you that the 'Stand Down' has been issued.		
15.	Attend debrief as required		

Code Black		Planning Staff – Mental Health Liaison		Action Card	
		Coordinator of Nursing, Mental Health		IMT 6	
Reporting to: Planning Officer					
Responsibilities: Mental health and staff welfare				Sign	Time
The mental health liaison role coordinates all mental health admissions, staff welfare, relative reception, volunteers and clergy					
Immediate Actions				Sign	Time
1.	On activation of the Incident Management Team (IMT) proceed directly to the Emergency Operations Centre (EOC)				
2.	On arrival at the EOC: - Obtain briefing from Planning Officer - Access Planning Officer ‘Box’ - Don Planning Staff – Mental Health Liaison blue tabard - Log into WebEOC using the generic logon and log in as Planning Staff in the EOC details tab - Commence WebEOC entry				
3.	Inform the following people of the incident and ask them to report to the EOC for a briefing				
	Consumer Liaison Officer ext. 1153				
	On-Call Pastoral Care (Switchboard for roster)				
	Work Health and Safety Consultant ext. 2462 / 2652				
4.	Provide briefing to: Consumer Liaison Officer, Social Workers and Pastoral Care team regarding the Relative Reception Plan The Work Health and Safety consultant regarding the “Staff Welfare Coordinator” role				
5.	Ascertain the number of staff likely to be required to implement Relative Reception Sub-Plan, and Staff Welfare plan, and arrange staffing as required				
6.	Ensure the designated relative reception area is prepared to receive relatives of casualties from the incident Note: This area will normally be the Bridson Room in the Mental Health Unit unless otherwise advised				
7.	Contact the Planning Officer to request refreshments to be made available in the designated relative reception area				
8.	Implement Relative Reception Sub-Plan as required				

9.	Ensure security arrangements are in place to keep Media away from relatives		
10.	Ensure relative reception area staff inform the EOC, on ext. 2599 when the relative reception area is ready to receive relatives		
11.	Liaise with Planning Officer regarding any issues arising from the Relative Reception Area		
12.	Ensure support structure for hospital staff is established, and staff have been informed how to access this support		
13.	Ensure all key activities, actions and decisions are documented in WebEOC and on the Samsung Flip and communicated with the Liaison Officers		
Intermediate Actions			
14.	Maintain liaison with the Planning Officer		
15.	Review the Relative Reception Sub-Plan, and Staff Welfare plan, and implement changes as required.		
16.	Ensure staffing plan is in place		
Extended Actions			
17.	Handover to subsequent shift		
18.	Maintain liaison with the Planning Officer		
19.	Review the Relative Reception Sub-Plan, and Staff Welfare plan, and implement changes as required.		
Stand Down / Recovery Actions			
20.	Ensure instructions are issued regarding how staff can access Support Structures post emergency		
21.	Once the Hospital Incident Commander has issued the 'Stand Down', inform your staff		
22.	Debrief staff involved in Relative Reception Sub-Plan and Staff Welfare plan		
23.	Return equipment to storage, as required		
24.	Participate in arranging a hot debrief under the direction of the Planning Officer and HIC		

Code Black		Logistics Officer		Action Card	
		Director Corporate Operations			
				IMT 7	
Reporting to: Hospital Incident Commander					
Responsibilities: Logistics, goods and services				Sign	Time
Manage and coordinate the provision of goods and services to the hospital. Staff that report to this role are as follows; - Logistics Staff (Coordinator of Nursing, Subacute and Kalamunda) - Logistics Staff (Director Allied Health)					
Immediate Actions				Sign	Time
1.	On activation of the Incident Management Team (IMT) proceed directly to the Emergency Operations Centre (EOC)				
2.	On arrival at the EOC: - Receive a briefing from the HIC - Access Logistics Officer 'Box' - Don Logistics Officer yellow tabard - Log into WebEOC using the generic logon and log in as the Logistics Officer in the EOC details tab - Commence WebEOC entry - Ensure your logistics staff have arrived - Delegate roles to your logistics staff				
3.	Brief the following areas/staff (as required), and request a status report within 15 minutes:				
	• Manager Patient Support Services				
	• HSS Liaison				
	• HSS Supply				
	• Campus Facilities Manager				
	• Security Supervisor				
4.	Liaise with the Manager Patient Support Services re allocation of PCAs to tasks as required by the EOC				

5.	Liaise with other areas under your command regarding resources (human and physical) required to manage the incident. Areas under command of the Logistics Officer are as follows; - Facilities and engineering - Communications and IT - Security - Patient transport		
	- Supply and stores - Catering - Linen - Patient Support Services		
6.	Liaise with Operations and Planning regarding resource requirements		
7.	Ensure all key activities, actions and decisions are documented in WebEOC and on the Samsung Flip and communicated with the Liaison Officers		
Intermediate Actions		Sign	Time
8.	Liaise with Operations and Planning Officers, and areas under your command regarding resource requirements		
9.	Ensure areas under your command receive briefings at regular intervals, and obtain status reports as required		
Extended Actions		Sign	Time
10.	Handover to subsequent shift		
11.	Liaise with Operations and Planning Officers, and areas under your command regarding resource requirements		
12.	Ensure people under your command are receiving regular briefings		
Stand Down / Recovery Actions		Sign	Time
13.	Ensure that workloads have returned to a pre-internal emergency or acceptable level.		
14.	Upon Stand Down, ensure that areas under your command are informed of the Stand Down order		
15.	Attend debrief as required		

Code Black		Operations Officer		Action Card	
		Director of Clinical Services or delegated operations cell staff member		IMT 8	
Reporting to: Hospital Incident Commander					
Responsibilities: Clinical services				Sign	Time
The Operations Officer is the person responsible to the HIC for the provision of clinical services in support of the incident. Staff that report to this role are as follows; • Operations Staff Surgical Patient Flow (Coordinator of Nursing, Surgical and Acute) • Operations Staff (Service Director, Mental Health) • Operations Staff (Coordinator of Nursing and Midwifery)					
Immediate Actions				Sign	Time
1.	On activation of the Incident Management Team (IMT) proceed directly to the Emergency Operations Centre (EOC)				
2.	On arrival at the EOC: • Receive a briefing from the HIC • Access Operations Officer 'box' • Don Operations Officer red tabard • Log into WebEOC using the generic logon and log in as the Operations Officer in the EOC details tab • Commence WebEOC entry • Ensure your Operations Staff have arrived • Delegate roles to your Operations Staff				
3.	If the Hospital Incident Commander is not present, assume that role until relieved by the Hospital Incident Commander				
4.	Confirm details of the Code Black:				
	Type of threat				
	Location of incident				
	Is police attendance required? – If yes, are they attending?				
	Is Psychiatry required? – If yes, are they attending?				
	Have any areas been evacuated, or require evacuation?				
	Does access to the area need to be restricted?				
	What impact is this incident having on hospital operations?				
	Is ambulance diversion required?				
	Type of threat				

5.	On arrival of your Operations Staff give a briefing: • The nature of the problem, • Any immediate critical issues • The initial plan of action Direct the Operations Staff to provide status reports in 15 minutes		
6.	• Utilise the Radio Operator (Liaison Officer) to liaise with the Emergency Scene Coordinator, to help determine:		
	• Impact on services		
	• Areas affected		
	• Resources required by the Emergency Response Team liaise with Logistics and Planning to arrange these resources		
7.	Ensure the Operations Staff have notified the areas under your responsibility of the incident (as appropriate) Areas of responsibility under the Operations Officer are as follows; • Emergency Department • Theatres / CSSD • ICU • Inpatient Wards • Outpatient Departments • Medical administration and staffing • Medical Imaging (PRC) • Pathology (PathWest) • Pharmacy • Mortuary		
8.	Provide the Hospital Incident Commander with updates on Operations Cell activities and issues		
9.	Ensure all key activities, actions and decisions are documented in WebEOC and on the Samsung Flip and communicated with the Liaison Officers		
Intermediate Actions		Sign	Time
10.	Liaise with the Emergency Scene Coordinator for: • Status reports from the incident scene • To ascertain requirements at the scene		
11.	Ensure regular updates are obtained from areas under your responsibility and communicated to the IMT		
12.	Liaise with Logistics and Planning to arrange resources as required		
13.	Continue to provide the Hospital Incident Commander with		

	updates on Operations Section activities and issues		
Extended Actions		Sign	Time
14.	Handover to subsequent shift		
15.	Ensure regular updates are obtained from areas under your responsibility and communicated to the IMT		
Stand Down / Recovery Actions		Sign	Time
16.	Ensure all clinical sectors of the hospital are functioning at pre-emergency, or acceptable levels		
17.	Provide the Hospital Incident Commander with information relevant to the decision to declare 'Stand Down'		
18.	Upon Stand Down, ensure that areas under your command are informed of the Stand Down order		
19.	Attend debrief as required		

Code Black		Operations Staff - Surgical Patient Flow		Action Card	
		Coordinator of Nursing, Surgical and Acute		IMT 9	
Reporting to: Operations Officer					
Responsibilities: Surgical activity				Sign	Time
The surgical patient flow role manages and coordinates all surgical activity					
Immediate Actions				Sign	Time
1.	On activation of the Incident Management Team (IMT) proceed directly to the Emergency Operations Centre (EOC)				
2.	On arrival at the EOC: • Receive a briefing from the Operations Officer • Access Operations Officer 'box' • Don Operations Staff Surgical Patient Flow red tabard • Log into WebEOC using the generic logon and log in as the Operations Staff in the EOC details tab • Ensure all key activities, actions and decisions are documented in WebEOC and on the Samsung Flip and communicated with the Liaison Officers				
3.	Brief the following areas/staff (as required), and request a status report within 15 minutes:				
	• Operating Theatre				
	• Same Day Unit				
	• CSSD				
4.	Liaise with the Planning Officer and gather current bed status from the Planning Staff Disaster Admissions / Outgoing Patient Flow				
5.	Assess current surgical activity and identify what priorities exist if elective surgery becomes limited or cancelled				
6.	Prepare theatres for possible influx of emergency surgery if there are injuries from the incident				
7.	Monitor bed status, liaise with Planning Staff Outgoing Patient Flow and coordinate new admissions from the Emergency Department to relevant wards				
8.	Liaise with the Operations Officer regarding resource requirements, and to provide situation reports				

9.	Monitor day surgery activity, liaise with the Operations Officer if there are disruptions to day surgery or it needs to be cancelled		
Intermediate Actions		Sign	Time
10.	Liaise with the Operations Officer regarding resource requirements, and to provide situation reports		
11.	Ensure people under your command are receiving regular briefings and updates		
12.	Monitor the activity and assess ongoing resource requirements for surgery and inpatient activity. Request additional resources via the Operations Officer or Planning Officer		
Extended Actions		Sign	Time
13.	Handover to subsequent shift		
14.	Liaise with the Operations Officer regarding resource requirements, and to provide situation reports		
15.	Ensure people under your command are receiving regular briefings and updates		
Stand Down / Recovery Actions		Sign	Time
16.	Ensure that workloads have returned to a pre-internal emergency or acceptable level		
17.	Upon Stand Down, ensure that areas under your command are informed of the Stand Down order		
18.	Attend debrief as required		

Code Black		Information Officer Coordinator Emergency Management	Action Card IMT 10
Reporting to: Hospital Incident Commander			
Responsibilities: Emergency Management subject matter expert			
			Sign
			Time
- Assist members of the Incident Management Team with issues regarding role and function - Assist Hospital Incident Commander with issues regarding hospital emergency procedures - Collate information regarding incident and ensure information distributed appropriately			
Immediate Actions			
			Sign
			Time
1.	On activation of the Incident Management Team (IMT) proceed directly to the Emergency Operations Centre (EOC) On arrival at the EOC: - Receive a briefing from the HIC - Access Information Officer 'box' - Don Information Officer green tabard - Ensure all key activities, actions and decisions are documented in WebEOC and on the Samsung Flip and communicated with the Liaison Officers		
2.	- Assist with making EOC Operational - Ensure all members of the IMT are in attendance, or are enroute - If not present within 5 minutes ask the Liaison Officer to contact the primary position holder - Ensure the Samsung Flip is operational		
3.	Log into WebEOC using the generic logon and log in as the Information Officer in the EOC details tab		
4.	Provide knowledge and resources to the Incident Management Team as requested		

5.	Gather information regarding the incident and present relevant information for whole IMT. e.g: maps, media reports, WebEOC situation reports Assist with: • Pop-up Messaging System (PMS) • Preparing global emails • Sending group paging messages via Esendex • Logging tasks and information on WebEOC		
	• Document impact assessment/s		
6.	Provide advice to the Hospital Incident Commander and Incident Management team as required		
Stand Down / Recovery Actions		Sign	Time
7.	Assist Hospital Incident Commander with issuing the 'Stand Down'		
8.	Assist with returning EOC to pre-emergency state, and restocking EOC resources in preparation for next activation		
9.	Finalise incident WebEOC report and present to the AKG EMC		
10.	Attend and assist with debrief		

Code Black	Liaison Officer 1 & 2 Office Hours - EA to DONM Office Hours - EA to ED Out of hours - PIC Clerk	Action Card IMT 11
Reporting to: Hospital Incident Commander		
Responsibilities: Clerical support	Sign	Time
To assist with clerical and record maintenance at the Emergency Operations Centre (EOC) during the emergency		
Immediate Actions	Sign	Time
1. On notification that the EOC is being activated for a Code Black proceed to the EOC On arrival at the EOC: - Assess Liaison Officer ‘box’ - Don Liaison Officer white tabard - Ensure all key activities, actions and decisions are documented in WebEOC and on the Samsung Flip and communicated with the Incident Management Team		
2. Obtain a briefing from the Hospital Incident Commander		
3. Log into WebEOC using the generic logon and log in as the Liaison Officer in the EOC details tab		
4. Ensure EOC attendance Log in WebEOC has been commenced, and is being completed as staff arrive and leave the EOC		
5. Commence an Operational Log of EOC activities/messages on WebEOC		
6. Set up the Samsung Flip and commence data entry		
7. Answer all incoming telephone calls to the EOC, and transfer calls to the appropriate person		
8. Monitor Fax machine for incoming communications – forward these to the relevant Officer. Send faxes as requested		
9. Relay messages as requested		
10. Assist the HIC with clerical duties as required/requested		

11.	If hand held radios have been deployed, establish radio communications with the Emergency Response Team, if not already done		
12.	Record radio communication details on an emergency response radio log sheet		
Stand Down / Recovery Actions		Sign	Time
13.	Collect any email or fax communication and collate the radio log		
14.	Save information on the Samsung Flip to a thumb drive		
15.	When stood down by the Hospital Incident Commander, inform normal supervisor of 'Stand Down'		
16.	Participate in the incident de-briefing process		

Code Black		EMHS Communications Officer Onsite Communications Officer (if available)	Action Card IMT 12
Reporting to: Hospital Incident Commander			
Responsibilities: Media and communications		Sign	Time
In consultation with the Hospital Incident Commander, manage media enquiries, media briefings, prepare media statements and staff information bulletins/globals			
Immediate Actions		Sign	Time
1.	On activation of the Incident Management Team (IMT) proceed to the Emergency Operations Centre (EOC)		
2.	On arrival at the EOC: • Receive a briefing from the HIC • Access Communications Officer 'box' • Don Communications Officer purple tabard • Ensure all key activities, actions and decisions are documented in WebEOC and on the Samsung Flip and communicated with the Liaison Officers / Incident Management Team		
3.	Develop public information and media statements, to be reviewed and approved by the Hospital Incident Commander prior to release If required: See Media Management Plan		
4.	Develop staff information bulletins, where required		
5.	Coordinate media briefings / releases with the DOH as required		
6.	Ensure copies of media statements and staff bulletins are provided to relevant staff		
7.	Document activities, actions and decisions in WebEOC – request an assistant to do this if required		
8.	In liaison with the Logistics Officer set up the media reception centre in Goline House Conference Centre		

9.	In accordance with health policy keep the media informed		
Stand Down / Recovery Actions		Sign	Time
10.	Submit any incident communication to EOC Liaison Officer		
11.	Attend debrief as required		

Code Black	HMA Incident Specialist As appropriate for incident	Action Card IMT 13
Reporting to: Hospital Incident Commander		
Responsibilities: Specialist agency advice	Sign	Time
Provide specialist advice regarding the incident to the Hospital Incident Commander and IMT		
Immediate Actions	Sign	Time
1. When requested by the Hospital Incident Commander proceed directly to the Emergency Operations Centre (EOC)		
2. Obtain a briefing from the Hospital Incident Commander regarding the incident and don HMA Incident Specialist black tabard		
3. Ascertain if the Hospital Incident Commander requires answers to specific questions, or a general briefing regarding the incident		
4. Prepare a briefing that addresses the Hospital Incident Commander's requirements from item 3. Deliver briefing		
5. Outline dangers, concerns, issues, that the hospital might face in relation to the incident. Advise on courses of action to mitigate these issues		
Stand Down / Recovery Actions	Sign	Time
6. Prepare briefing and report on risks, issues or concerns that may be encountered in the recovery phase. Advise on courses of action to mitigate these issues.		
7. Attend debrief if requested		

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